Protocol for genitourinary symptoms of menopause (GSM) Using Low-Intensity Shockwaves



Device: MoreNova - Li50F Linear Transducer

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1. Purpose and Scope

This protocol provides a standardized treatment plan for using low-intensity shockwaves to manage Vaginal Atrophy / Genitourinary Syndrome of Menopause (GSM). It is intended for healthcare providers using the MoreNova Device with the Li50F Transducer to target the Vaginal wall – 6 angles (60° apart).

This protocol applies to women experiencing genitourinary symptoms of menopause (GSM), including vaginal dryness, burning, or irritation. For further information on clinical studies and efficacy, visit our Clinical Data Center.

Device Overview:

MoreNova is a portable Electromagnetic (EM) Low-Intensity Shockwave (LISW) device designed to reduce inflammation, dissolve calcifications, and stimulate angiogenesis, improving outcomes for various conditions in men and women.

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2. Introduction

Low-Intensity Extracorporeal Shockwave Therapy (LiESWT) is a non-invasive technology that uses acoustic waves to promote vascularization, enhance blood circulation, and stimulate nerve regeneration. It has been clinically shown to improve genitourinary menopausal symptoms by targeting key vaginal and periurethral tissues involved in mucosal health, elasticity, vascularization, and tissue regeneration.

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3. Background Condition Overview:

- Genitourinary Syndrome of Menopause (GSM) is a chronic condition associated with estrogen deficiency, affecting the vagina, vulva, urethra, and bladder.
- It is characterized by vaginal dryness, burning, irritation, dyspareunia, and urinary symptoms such as urgency, dysuria, and recurrent urinary tract infections.
- These symptoms result from thinning of the urogenital epithelium, reduced vascularization, decreased elasticity, and altered tissue integrity following menopause.

Mechanism of Action:

- Low-intensity shockwave therapy is thought to stimulate biological responses including neovascularization, improved microcirculation, and enhanced tissue oxygenation in the treated vaginal and periurethral tissues.
- These effects may support tissue regeneration, improve mucosal health and elasticity, and thereby contribute to relief of vaginal and urinary symptoms associated with GSM

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4. Patient Selection Criteria Inclusion Criteria:

- Females aged 40–75
- Postmenopausal women diagnosed with vaginal atrophy or GSM
- Experiencing symptoms such as vaginal dryness, irritation, itching, burning, or painful intercourse (dyspareunia)
- Symptoms persisting for ≥3 months despite use of moisturizers or lubricants

Exclusion Criteria:

- Active vaginal or urinary tract infection.
- Undiagnosed vaginal bleeding.
- History of estrogen-dependent malignancy (e.g., breast or endometrial cancer).
- Recent pelvic surgery or radiation therapy (<6 months).
- Pregnancy or lactation.
- Presence of implanted electronic or metal devices sensitive to electromagnetic radiation.
- Severe vaginal prolapse or anatomical distortion preventing proper Transducer placement.

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5. **Pre-Treatment Screening**

To determine the patient's suitability for LiESWT, conduct the following:

- Comprehensive medical and gynecologic history, including menopausal status, hormone therapy use, and urinary symptoms.
- Pelvic and genital examination to evaluate vaginal mucosa, tissue integrity, and signs of atrophy or inflammation.
- Diagnostic evaluation as indicated to exclude active infections, unexplained bleeding, or structural/anatomical abnormalities of the genitourinary tract.

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6. **Treatment Protocol Preparation**:

- Confirm findings from a comprehensive medical history and gynecological examination, with attention to menopausal status and genitourinary symptoms.

- Counsel the patient on the treatment procedure, anticipated benefits for GSM symptoms, potential risks or discomfort, alternative treatment options, and realistic expectations regarding outcomes.

Procedure:

- **Number of Sessions**: 6 sessions
- **Session Frequency: Twice per week.**
- **Session Duration**: 25 minutes.
- **Applications Per Session**: Vaginal wall – 6 angles (60° apart).
- **Shocks Per Application**: 500.
- **Total Shocks Per Session**: 3000.

Treatment Parameters:

- **Energy (HV) Settings**: 2.0 kV.
- **Frequency (FRQ)**: 2 Hz.

Post-Treatment Care:

- Advise patients to avoid vaginal intercourse and intravaginal products for 24 hours following each treatment session.
- Encourage adequate hydration and instruct patients to report any unexpected discomfort, irritation, bleeding, or other adverse events to their healthcare provider.

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7. Treatment Areas and Techniques

The treatment focuses on the following anatomical areas:

- Vaginal canal (Vaginal wall)

  The rotation pattern:

  - -180
  - -120
  - -60
  - 0
  - 60
  - 120
  - 180

Transducer Placement:

- Apply the Transducer gently to the designated areas, using conductive gel to facilitate energy transfer.
- Gentle internal application with continuous coupling gel, along the vaginal wall, rotating the transducer to six angles
- Deliver 500 shocks to each application area, following the predefined sequence.

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8. Patient Comfort and Safety

To ensure patient comfort and safety during the procedure:

- Position the patient in a lithotomy or supine position for optimal access.
- Use conductive gel to minimize discomfort and enhance shockwave transmission.
- Continuously monitor patient feedback during the session.
- Adjust settings or pause treatment if discomfort is reported.

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9. Treatment Outcomes and Follow-Up

Expected Results:

Patients may begin to experience gradual improvement in genitourinary symptoms of menopause, such as vaginal dryness, discomfort, or dyspareunia, within several weeks (approximately 3–6 weeks) after initiating treatment. Ongoing clinical follow-up and symptom-based assessments are recommended to monitor response and guide further management.

Follow-Up Visits:

Schedule bi-weekly follow-ups during the treatment period to monitor outcomes and address concerns.

Long-Term Maintenance:

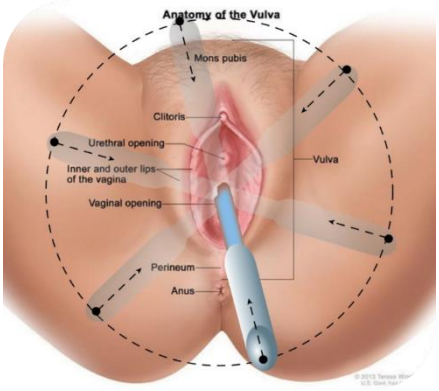
Consider scheduling maintenance sessions every 6 months to sustain benefits.

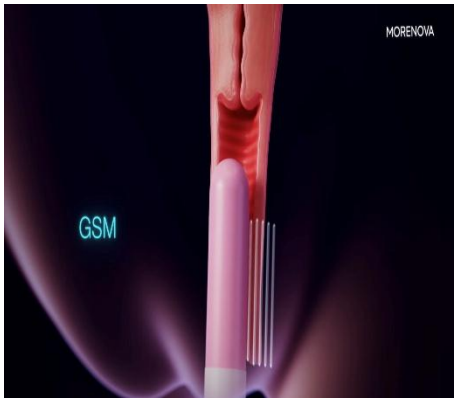
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10. Visuals and Diagrams Li50F



- **Transducer placement technique and anatomical areas targeted (Vaginal canal, Vaginal wall).**
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11. **Quality Control Device Calibration**:

- Verify device settings before each session.
- MoreNova Device recognizes preprogrammed parameters to ensure accuracy.

Clinician Training

Ensure all operators are trained to use the MoreNova Device and the Li50F Transducer.

- Clinicians should be familiar with this protocol and the "Instructions for Use" provided by the manufacturer.

Checklist for Each Session:

- Confirm that the patient meets the inclusion criteria and has no contraindications.
- Ensure the device and Transducer are clean, sanitized, and function correctly.
- Verify that the Transducer placement aligns with the specified anatomical area.

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12. **References**

For clinical studies and additional resources, visit the Initia Clinical Data Center.